

**Methods:** This is a hospital-based, cross sectional descriptive study. All TNBC cases attending Medical Oncology Department, YGH, were collected from January 2019 to December 2019. Immunohistochemistry (IHC) for androgen receptor was done by peroxidase anti-peroxidase method.

**Results:** Among 808 breast cancer patients attended during study period, 515 patients were available for IHC testing and 88 patients were TNBC subtype. In those patients, 61 TNBC cases were included in this study; 22 patients were AR positive and 39 patients were AR negative. In TNBC, the youngest was 33, the eldest was 73 and median age was 53. Nine patients were <40 years of age. In histology, invasive ductal carcinoma was the commonest (91.8%). Regarding staging, unlike developed countries, advanced stage (64%) was the most common stage. Among AR positive patients, 23% were <40 years of age and 77% were ≥40 years. sixty eight percents of patients presented in advanced stage. In patients with negative AR status, 10% were <40 years of age and 90% were ≥40 years of age. Regarding the stage, 38% were early stage and 62% were advanced stage.

**Conclusions:** Although the risk of breast cancer increases with age, many younger patients were included in this study, reflecting that the trend is changing. Most patients presented at late stage and this points out that our population have limited knowledge about breast cancer. In this study, AR negativity (64%) is much more than that of positivity (36%), there were much younger patients in AR positive subgroup and not much association was seen between AR and histology. This study shows fundamental knowledge regarding AR status in TNBC in Myanmar.

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#### 450P Are grade III well-differentiated neuroendocrine tumours a unique clinic-pathological entity?

J. Kalvala<sup>1</sup>, M. Wood<sup>1</sup>, A.R. Rao<sup>2</sup>, A. Arora<sup>3</sup>, J. Adhikaree<sup>4</sup>

<sup>1</sup>School of Medicine, School of Medicine - University of Nottingham, Nottingham, UK; <sup>2</sup>Oncology Department, Nottingham City Hospital, Nottingham, UK; <sup>3</sup>Oncology Dept., City Hospital Campus - Nottingham University Hospitals NHS Trust, Nottingham, UK; <sup>4</sup>Academic Oncology Department, Nottingham City Hospital, Nottingham, UK

**Background:** Neuroendocrine neoplasms (NENs) are a rare group of malignancies with significant mortality. Recently, Grade 3 (G3) NENs have been dichotomised into well-differentiated neuroendocrine tumours (NETs) and poorly differentiated carcinomas (NECs), with seemingly different characteristics. Treatment for these groups differs, although there is not enough evidence to identify a uniform standard of care. We aim to understand whether the difference between NET and NEC G3 is clinically significant, and if so, what these are in terms of their clinical characteristics.

**Methods:** A retrospective cohort study was done to identify patients diagnosed with NEN G3 in Nottinghamshire. Patients were included if they were diagnosed after 2017 with Ki-67 score of >20%, which is metastatic and not amenable to curative treatment. Patients with neoplasms that exhibit large- or small-cell like morphology were excluded. Clinical characteristics and treatment were collected and analysed using Kaplan-Meier curves and ANOVA.

**Results:** 22 patients were included. The mean overall survival (OS) of the cohort was 778 days and the median OS was 461 days. Median OS for patients with Ki-67 score ≥40% was 861 days, as compared to 217 days for patients with a score <40% (p=0.078). Median OS for patients with unknown primary site was worse, followed by pancreatic (217 days vs 461 days; p=0.939). Mean first-line progression free survival (PFS) for patients who did not receive platinum-based chemotherapy was numerically superior than for patients who did receive this (449 vs 127 days; p<0.001). Mean first-line PFS for patients who received alkylating-based chemotherapy was higher than for patients who did not receive this (228.22 vs 156.46 days; p=0.003). Capecitabine and temozolomide treatment had the best objective response rate at 54.5%.

**Conclusions:** This study described the histopathological and clinical factors of high-grade NENs. These findings contribute to a better understanding of the prognostic significance of factors in this group of neoplasms, sufficient to rationalise a difference in care for patients with G3 NETs and true high-grade NECs.

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#### 451P The activity and safety of anlotinib combined with chemotherapy for patients with desmoid tumor: A retrospective study in a single institution

P. Zhang, X. Liang, X. Niu, S. Gao, X. Wang, J. Wang, W. Yao

Department of Bone and Soft Tissue, Henan Cancer Hospital/Affiliated Cancer Hospital of Zhengzhou University, Zhengzhou, China

**Background:** Desmoid tumor (DT) is an aggressive fibroblastic neoplasm with a high propensity for local recurrence. Although multiple therapeutic modalities seem effective for DT, the standard systemic treatment for symptomatic and progressive DT remains controversial. As targeted therapy, tyrosine kinase inhibitors have been recently reported to contribute to the treatment of DT. Thus, the purpose of this study was to assess the efficacy and safety of chemotherapy anlotinib, a novel multi-kinase angiogenesis inhibitor, in patients with DT.

**Methods:** This retrospective, single-arm clinical trial recruited patients with desmoid tumor who received first-line therap. Patients received anlotinib and methotrexate plus vinorelbine until 12m or disease progression (PD). The primary endpoint was 6m PFS%. Secondary endpoints are PFS, OS, objective response (ORR), disease control rate (DCR) and safety. Tumor response was assessed by the Response Evaluation Criteria in Solid Tumors 1.1 criteria.

**Results:** Between April 2020 and June 2022, 13 patients were enrolled and 11 of them have received at least one tumor assessment. The median age was 34 (range: 10-67) years, 4 males (30.8%) and 9 females (69.2%). 1 (9.1%) patients achieved a complete response, 2 (18.2%) patients achieved a partial response and 8 patients (72.7%) achieved disease stability. The 6-month PFS rates were 100%. The disease control rate was 100% (11/11) and the objective response rate was 27.3% (3/11). The most common AEs were hypertension (18.2%), leukopenia (18.2%). No ≥grade 3 TRAE was recorded.

**Conclusions:** Anlotinib combine with chemotherapy was effective against DT with an acceptable safety profile, and significantly slowed the disease progression. Further, multicenter studies with a longer follow-up time are needed to characterize fully the clinical application of anlotinib combine with chemotherapy in DT.

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#### 452P Medical and paramedical students' awareness of environmental risk factors for cancer

M.B. Behery<sup>1</sup>, M.A. Elsayed<sup>2</sup>, E. Elshebiny<sup>3</sup>

<sup>1</sup>Undergraduate, MFM - Menoufia Faculty of Medicine - Student Research Program (SRP), Shebeen El-Kom, Egypt; <sup>2</sup>Undergraduate not specialized yet, MFM - Menoufia Faculty of Medicine - Student Research Program (SRP), Shebeen El-Kom, Egypt; <sup>3</sup>Internal medicine/ Rheumatology & Immunology unit, MFM - Menoufia University - Faculty of Medicine, Shebeen El-Kom, Egypt

**Background:** Cancer is a serious disease, associated with significant morbidity and mortality. Despite the high burden of cancer, many risk factors have been linked to carcinogenesis. Some of them are preventable and with good awareness, they could be one of our tools in relieving that burden. Our study aimed to assess medical and paramedical students' awareness of the environmental risk factors for cancer.

**Methods:** A web-based cross-sectional study was conducted among medical and para-medical students of Menoufia University. The data were collected from April to May 2022. We used a pre-validated questionnaire that was designed to address knowledge about different environmental risk factors for cancer.

**Results:** We obtained responses from 251 participants. The majority of respondents were medical students (n= 204; 81.3%), males (n= 157; 62.5%), and living in rural areas (n=160; 63.7%). Most participants (n=173; 68.9%) had a knowledge score of more than 50% with an average score of 24 out of 43 (24.22 ± 10.7). There was no Knowledge difference between medical and paramedical students (n=144; 70.6% vs n=29; 61.7%; p=0.237). Most students recognized radiation, smoking, and alcohol as risk factors for cancer (76.9%, 73.7%, and 70.7%). Regarding Vit D deficiency, paramedical students were more aware than medical students (51% vs 33.3%; p=0.023). However, a lower percentage acknowledged high red meat intake and low fiber diet (29.5%) as potential contributors to the occurrence of cancer.

**Conclusions:** There was good knowledge among medical and paramedical students about most of the environmental risk factors related to cancer pathogenesis. However, there was less awareness of diet-related ones, especially those related to colorectal cancer.

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